

Integrating with HealthEquity to make it easier to use HSA/FSA funds

30% of Americans have a health savings account, but many forget these funds exist. To reduce friction and make it easier to pay medical bills, Cedar integrated with HealthEquity, one of the largest HSA/FSA banks in the country. I designed the end-to-end patient experience.

The situation

Cedar already had an integration with Highmark, an insurance company, that showed patients their HSA/FSA balance alongside their medical bill. The results were promising: a **33% relative increase in HSA/FSA payments**, and an **8% collections lift** among patients with HSA/FSA funds. The problem was reach. That integration only worked for a couple of providers, and only for patients whose HSA/FSA happened to run through Highmark.

Those results were the strategic case for going further. If surfacing a balance moved the needle this much, the win wasn't refining the feature — it was expanding who could get it. That meant adding integration partners beyond a single insurer. I led the push to integrate with HealthEquity, the largest HSA administrator in the country at 22% of the market, and designed the end-to-end experience: patients activating their HealthEquity account and seeing their balance so they could pay their bill more easily.

Identifying the opportunity

~**33%** Of Americans have an HSA or FSA account

22% Of the HSA/FSA market held by HealthEquity — the largest administrator in the country

33% Relative increase in HSA/FSA payments when Cedar showed patients their balance

Expanding reach meant taking on a new problem: activation. With Highmark, the balance came from the payer, who already had a data relationship with the patient. Connecting straight to HealthEquity meant Cedar didn't have that relationship yet — we couldn't just pull a patient's balance and show it. Patients had to actively link their account and consent to share it first. Designing that activation moment, not the balance display itself, became the real design problem.

Building the experience

We couldn't build everything at once, so I scoped the MVP around where the value was most likely to show up first. I designed the end-to-end journey, and we focused on three pieces:

- **Activation and consent** — the biggest new source of friction. With HealthEquity, we couldn't just pull a patient's account in automatically the way we could with Highmark. Patients had to actively consent first, so getting that moment right was priority one.
- **Showing funds alongside the bill** — the Highmark results already told us this drove value. The balance needed to sit right where the payment decision happens, not buried somewhere else.
- **Funds utilization** — once a patient could see their balance, the next question was how to actually get them to use it. Billing reminders were one lever here: folding the balance into email and paper statements to catch patients before they opened the app.
(placeholder — more to add here)

Testing it: MVP launch at Providence

We launched a 50/50 A/B test at Providence — 19k patients in test, 19k in control.

+2.4% more patients reached the Cedar Pay app **+5.4%** increase in collection rate — roughly **\$1.5M/year for Providence alone**, even though only ~4% of guarantors there had a HealthEquity account

Only 13% of engaged patients actually gave consent to view their balance inside the app. Most of the lift was coming from the *nudge in the email*, not the in-app feature itself — patients didn't need to see the balance to be motivated by knowing it existed. That shaped where I focused next.

Iterating and improving

Two threads followed from the launch: getting more patients to actually use the funds they could now see, and getting more patients through activation in the first place.

Why activation lagged. We had two working theories for why more patients didn't link their account right away. Some HSA holders use the account as an investment vehicle rather than a spending account — that's not behavior we can change, so some leakage was always going to be there no matter how we designed the flow. The more useful theory, and the one we could actually act on, was timing: patients don't necessarily want to pay the moment a bill arrives. Some want to understand their insurance coverage first, some want to dispute the bill, and for patients who do intend to use their HSA/FSA funds, activation is more likely once they're

actually ready to pay rather than the moment the bill lands. That's the group we designed around.

FSA expiration email. The FSA expiration nudge was one direct attempt to act on that second group — patients who intend to use their funds but just aren't ready yet. Expiring funds gave us a built-in reason to create urgency instead of waiting for them to come around on their own. Unused FSA funds expire at year-end, so I ran an experiment: could reminding patients their funds were about to disappear get them to pay? For patients we knew had an FSA, a targeted expiration email drove a **10% increase in payment rate**. For patients we didn't know had an FSA, a generic "your bill is ready" email actually outperformed an FSA-specific nudge — the FSA language read as a distraction, not a motivator, for people who might not have one. One client, Hartford, saw a **24% increase in engagement** and **16% increase in payments** just from the generic email, which we traced to timing: Hartford's communication cadence is slower than other clients, and this email landed right before the holidays. The takeaway shaped a clear rule going forward — target FSA expiration messaging only at patients we know have an FSA, and look for ways to infer FSA status for everyone else instead of guessing.

Activation nudge copy. The piece I actually rewrote was the callout on the bill's homepage — the prompt that gets a patient into the linking flow in the first place. The original copy was framed as a question: "Do you have a HealthEquity HSA or FSA? Link your account." I wanted something more direct — ideally "You have funds — view funds" — but Legal pushed back, since we couldn't tell a patient they had funds until they'd actually given consent to let us check. The compromise was "Use your HSA or FSA on your bills. Check my balance." — still direct and attractive as a value prop, without claiming something we couldn't yet confirm. Around the same time as this change, click-through into the confirmation page went up, from roughly 8-10% to 10-12%. But the confirmation rate itself dropped, from around 90% to 85-87%, likely tied to a separate copy change on the confirmation screen itself made in the same release. Getting people to the door and getting them through it pulled in opposite directions — a tradeoff I'm still calibrating.

Across these changes, we did see more activation and more utilization over time — though it's hard to cleanly separate how much came from the updates themselves versus patients simply reaching that point in their billing cycle where they were ready to view their funds and put them toward a bill. Realistically, it was some combination of both.

Scaling it into a system

The MVP proved the model, so I focused the second half of this project on making it repeatable — and on closing gaps that surfaced once real patients started using it.

On the engineering side, this meant rebuilding a lot of the underlying infrastructure into a shared **HBA Service** — compatible with our existing HSA/FSA integrations and built to scale to new ones we hadn't onboarded yet, rather than a bespoke build every time we added a vendor. That

included enabling **cross-provider consent**, so a patient who links their HealthEquity account at one provider automatically sees that balance at every other Cedar provider too.

On the design side, my job was making the experience hold together across integrations that don't actually behave the same way. I built reusable components, standardized how we show a balance, and designed error states for the different ways a connection can fail. The harder part was mapping what each integration type actually needs, because they sit on a real spectrum:

- **Highmark** — the seamless end. We already know a patient has a balance and can pull it in directly, no patient action required.
- **HealthEquity** — the middle. We know the patient has a balance, but need their consent before we can show it.
- **Plaid** (a later integration I also worked on) — the hard end. We don't know upfront whether a patient has a balance at all, so the experience has to work without that certainty.

Designing one system that flexes across "we know and can just show it" to "we have no idea and have to find out" was the real design system work here — not just polishing the balance card itself.

I also wrote documentation and decks so the value of the feature — and the reasoning behind it — didn't live only in my head.

Two more fixes came directly from patients, not from the metrics.

Nicknames. Patient survey comments kept surfacing the same confusion: we displayed cards by their last four digits, and patients with more than one HSA/FSA account couldn't tell them apart. I designed the ability for patients to nickname their cards when they linked them — a small fix, but one that came straight from listening to the people actually using the feature rather than from a dashboard.

Self-serve revoke. Removing account access originally required a phone call and a manual fix from engineering — slow for the patient and a needless tax on the team. I designed a self-serve setting so patients could revoke access themselves, closing the loop on the consent model instead of treating it as one-directional.

Activation modal, updated for legal. As we scaled to more clients, Legal required clearer disclosure on exactly what the consent screen was asking patients to agree to. I redesigned the activation flow to add a "Learn more" FAQ modal alongside the consent screen — spelling out what data a provider could see and which bills actually qualify for HSA/FSA — so the compliance requirement doubled as a chance to make the ask itself less confusing.

The outcome

+5.4% Collection rate lift at Providence, ~\$1.5M/year at just one client

+2.4% More patients reaching the Cedar portal, driven mostly by the email nudge

+10% Payment rate lift from targeted FSA expiration emails

16% Of patients shifted to paying with their HBA card instead of another method

Given the strength of the Providence results, Data Science recommended rolling HealthEquity's HBA features out to every eligible client — which we did, alongside a roadmap of enhancements (click-to-pay, deeper personalization by bill size, expanding beyond HealthEquity to cover more of the HSA/FSA market).